# Skanda Radiology

RADIOLOGY REPORT — AI-ASSISTED DRAFT

Report date: 10/2/2026 · Accession: 02D0D80CEF

| | | | |
|---|---|---|---|
| Patient Name | — | Patient ID | — |
| Age / Sex | — | Study Date | 2026-10-02 |
| Referring Physician | — | | |

## CLINICAL INDICATION / HISTORY

—

## EXAM

Radiograph (X-ray) · Chest · PA

## KEY IMAGES

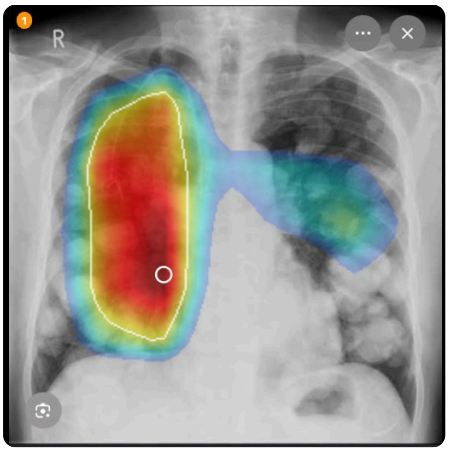



1. **Multiple bilateral pulmonary nodules** · Bilateral lungs · not localized
2. **Increased bronchovascular markings / perihilar opacity** · Perihilar regions, bilateral · not localized

*Key 1 — user-sample.jpg*

## CLASSIFIER HEATMAPS

One map per finding: where an independent image classifier (TorchXRayVision) found evidence for that finding type, within the finding's own region (its box, or the zone it names). Red = strongest, outlined = hot region, ring = peak; the box is the reading model's. "Regional evidence" is the strongest value in the region relative to the type's peak anywhere on the film: low means the classifier sees little at that location. Coarse: on held-out radiologist-labelled films the peak fell inside the lesion 17% of the time. A prompt to look, not a localisation.



**1 Multiple bilateral pulmonary nodules**
nodule / mass · classifier 80% · supported · whole film (no region given)



**2 Increased bronchovascular markings / perihilar opacity**
opacity · classifier 82% · supported · regional evidence 99%

## TECHNIQUE

PA Chest Radiograph

## COMPARISON

None available.

## FINDINGS

Multiple bilateral pulmonary nodules are identified throughout both lung fields, varying in size but appearing relatively well-defined and round. These findings warrant further investigation.

There is increased prominence of bronchovascular markings bilaterally, particularly noted in the perihilar regions, which may suggest interstitial changes or airway inflammation/edema.

The heart size appears within normal limits.

The mediastinum is midline.

No pleural effusion or pneumothorax is identified.

No acute osseous abnormalities are seen.

## IMPRESSION

1. **Presence of multiple bilateral pulmonary nodules requires further evaluation to determine etiology (e.g., metastatic disease, granulomatous infection, hamartomas).**
2. **Increased bronchovascular markings/perihilar opacity may represent interstitial changes or airway inflammation/edema and warrants clinical correlation.**

## RECOMMENDATIONS

Consider a CT chest examination for further characterization of the pulmonary nodules.

Clinical correlation is recommended to evaluate the significance of the increased bronchovascular markings in the context of the patient's history and presentation.

## AI FINDINGS SUMMARY

| # | FINDING | LOCATION | SEVERITY | BENCHMARK | MODEL-STATED (UNCALIBRATED) |
|---|---|---|---|---|---|
| 1 | Multiple bilateral pulmonary nodules | Bilateral lungs | Significant | 7/13 | 95% |
| 2 | Increased bronchovascular markings / perihilar opacity | Perihilar regions, bilateral | Moderate | 29/38 | 60% |

Benchmark: how many calls of this kind were confirmed by reference labels in the local benchmark. Model-stated confidence is the model's own figure and is not calibrated.

**UNSIGNED — PRELIMINARY AI DRAFT**

Awaiting radiologist review and signature

AI-generated decision support; must be verified by a qualified radiologist. Not a diagnosis. Drafted with AI assistance (MedGemma 27B, run locally).